



FOLLOW-UP VISIT — 09/21/2023

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 09/21/2023
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PAIN TODAY (0-10)	7/10
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Two-week post-op follow-up. Surgical staples removed. Wounds are healing cleanly without erythema. Distal sensation is still diminished in the radial distribution, but showing slight subjective improvement. Left wrist extension is weak (3/5). Refer to immediate outpatient physical therapy 3x/week for active mobilization of left shoulder, elbow, wrist, and neck.

Reviewed medications, function, and goals. Time spent: 25 minutes; >50% counseling/coordination.

Dr. Myrtle Turner

Dr. Myrtle Turner, MD, FACS — Orthopedic Surgery

NPI 1442039981 · Signed 09/21/2023

FOLLOW-UP VISIT — 10/19/2023

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 10/19/2023
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PAINTODAY (0-10) (0)	5/1010
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Six-week post-op follow-up. Left upper extremity X-ray shows early callus formation around the humerus and distal radius plates. Patient is compliant with PT at Lang-Weimann. Still experiencing significant neck stiffness and pain (5/10). Left hand grip strength remains weak (4/5).

Reviewed medications, function, and goals. Time spent: 25 minutes, >50% counseling/coordination.

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NPI 1442039981 · Signed 10/19/2023

FOLLOW-UP VISIT — 11/16/2023

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 11/16/2023
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PAIN TODAY (0-10)	4/10
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Ten-week post-op follow-up. Patient complains of worsening left arm aching and severe neck pain. Mentions he had to miss several physical therapy sessions recently. Stressing the extreme importance of continuous physical therapy to prevent contractures and muscle wasting.

Reviewed medications, function, and goals. Time spent: 25 minutes, >50% counseling/coordination.

Dr. Myrtle Turner

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NPI: 1442039981 Signed: 11/16/2023

FOLLOW-UP VISIT — 12/14/2023

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 12/14/2023
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PAIN TODAY (0-10)

6/10

Four-month post-op follow-up. Fracture lines are nearly consolidated. However, there is persistent restriction in the range of motion of the left elbow and wrist. Sensation in radial nerve distribution remains decreased. Patient complains of worsening neck pain. Ordering follow-up MRI of the left upper extremity and C-Spine to assess soft tissue integrity and neural compression.

Reviewed medications, function, and goals. Time spent: 25 minutes, >50% counseling/coordination.

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NPI 1442039981 · Signed 12/14/2023

FOLLOW-UP VISIT — 02/08/2024

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 02/08/2024
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PAIN TODAY (0-10)	3/10
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Post-op follow-up. Patient has transitioned PT care to Emory Sports Medicine Complex. Left arm bony fusion is fully complete. Residual left forearm pronation and supination are limited. Left shoulder range of motion remains restricted. Cervical range of motion is moderately limited.

Reviewed medications, function, and goals. Time spent: 25 minutes, >50% counseling/coordination.

Dr. Myrtle Turner

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NPI 1442039981 · Signed 02/08/2024

FOLLOW-UP VISIT — 03/14/2024

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 03/14/2024
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PAIN TODAY (0-10)	3/10
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Final clinical orthopedic assessment. Incisions are maturely scarred. Bony healing is stable and robust. However, patient has reached maximum medical improvement (MMI). Significant permanent disability of the left arm is noted, with persistent loss of elbow flexion/extension (15 degrees deficit) and wrist supination (20 degrees deficit). Sensation over the left radial aspect of the hand remains permanently blunted. Persistent mild-to-moderate chronic neck pain continues.

Reviewed medications, function, and goals. Time spent: 25 minutes, >50% counseling/coordination.

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NPI 1442039981 · Signed 03/14/2024



DIAGNOSTIC IMAGING REPORT

PATIENT Dax Vance	DOB 04/12/2001	MRN 940-2281-A	DATE OF SERVICE 12/14/2023
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EXAMINATION	X-ray Left Arm & Forearm
ACCESSION	RAD-2023-9988 · Exam date 12/14/2023
INTERPRETING MD	Dr. Myrtle Turner, MD, FACS

TECHNIQUE

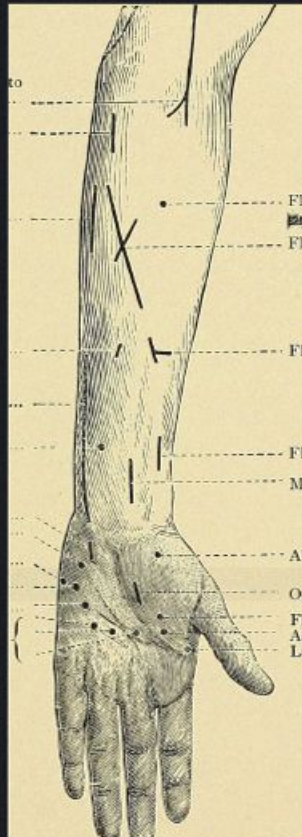
Frontal and lateral views of left humerus and left forearm.

FINDINGS

The Dynamic Compression Plate across the left humerus shaft is in stable position with no signs of lucency, backing out of screws, or failure. There is excellent bony bridging callus across the fracture line. Volar locking plate in the distal radius is intact with no evidence of displacement. Distal radioulnar joint alignment is maintained.

IMPRESSION

Well-healed left humerus and left distal radius fractures with stable internal hardware.



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